

AT A GLANCE

- **First IV ketamine session at Royal Oak yesterday (5/18).** Session itself was difficult — shaking, crying out. Mom: "terrible, convulsive."
- **Afterward Dad was more coherent than Mom has seen him in a long time** — clear, measured, articulate. Dad himself said he didn't feel better.
- **Royal Oak: off the table.**
- **Ann Arbor clinic transfer:** first available appointment in early June (~2–3 weeks). A SSRI/SNRI switch would delay it by a few days.
- **CA option:** off the table per Dad.
- **Dad confirmed he did not take the Seroquel** that was proposed last weekend.
- **Dr. Rubin proposed a medication restructuring:** Cymbalta → Lexapro switch; add lemborexant; add depakote; remove trazodone and doxepin. Lemborexant came from Julane.
- **Exact recommendations from each provider are still unclear.** Dr. R and Dad spoke this morning; Dad then emailed Dr. R; follow-up not visible yet. What Dr. Chilton and Julane specifically endorsed is also not yet confirmed.
- **February 2026 course was IV ketamine** (not sublingual) and produced a dramatic response. Later lozenge use was good but less transformative. Dad is specifically an IV-ketamine responder.
- **Dad has a call with Julane this afternoon.**

WHAT WE KNOW

- **5/18 session at Royal Oak:** difficult experience in the chair (shaking, crying out). Dad described the commute as "going to Africa." No provider in the room during the session.
- **Post-session functional state on 5/18:** "more coherent than Mom has seen him in ages — clear, measured, articulate." Dad said he didn't feel better.
- **Mom used the word "retraumatizing."**
- **Dad did not take the proposed quetiapine.** Prior context in [handout #18 — Quetiapine \(Seroquel\) low-dose proposal](#).
- **Provider geography:** Royal Oak off the table; CA off the table per Dad; A2 the operative path; first A2 appointment early June.
- **Dad's confirmed ketamine response is to IV specifically** (Feb 2026 course). Lozenges produced good but less transformative results. This is a specific signal that protects the value of getting back to IV.
- **Current sleep stack:** doxepin 6 mg HS, trazodone 50 mg PRN HS, melatonin 3 mg HS. Chilton-cleared. See [handout #13 — Doxepin titration](#) and [handout #19 — NPI sleep additions](#).
- **Current antidepressant:** duloxetine 60 mg, on board since 11/2025 (~6.5 months).

WHAT WE DON'T KNOW YET

- **Exactly what Dr. Rubin is recommending.** Dr. R and Dad spoke this morning. Dad then emailed Dr. R. The reply / what was settled is not yet visible.
- **Exactly what Dr. Chilton recommended.** "Less acute anxiety before next session" is the stated clinical goal. Specific medication changes Dr. R is operationalizing (SSRI switch, depakote add, removal of doxepin/trazodone) are not confirmed as Chilton-endorsed.

- **Exactly what Julane recommended.** Lemborexant came from her. Anything else she said on meds or on yesterday's session is not yet confirmed.
- **Whether the post-session coherence persisted into the evening / today,** or faded.
- **What an Ann Arbor clinic intake looks like operationally** — protocol, room arrangement, in-room provider presence, pre-session anxiolysis protocol.
- **Whether Dad is open to a single pre-session benzodiazepine dose** as part of the A2 intake.

FOLLOW-UPS NEEDED

1. **Get Dr. Rubin's specific written recommendation in writing.** Once the email exchange concludes, capture the exact proposed plan and the timing.
2. **Ask Dr. Chilton directly:** *"Did you specifically endorse the Cymbalta→Lexapro switch and the depakote add, or did you raise pre-session anxiety as the clinical goal and leave operationalization to Dr. R?"*
3. **Get Julane's written read of yesterday's session.** Distinguishing abreactive integration from intolerable distress is her domain.
4. **Confirm Ann Arbor clinic intake details:** date, protocol, in-room provider, pre-session anxiolysis approach.
5. **Verify Dad's caffeine intake directly with Dad** rather than via second-hand report. See [handout #19, Tier A items](#).
6. **After the Julane call: capture what was said** about yesterday's session and about the proposed medication plan.

CONCERNS ABOUT THE PROPOSED PLAN

Cross-referenced to prior research where relevant.

- **The Cymbalta → Lexapro switch contradicts the stated goal of reducing acute pre-session anxiety.** Duloxetine discontinuation produces acute anxiety, dizziness, paresthesias, nausea, agitation — introducing acute anxiety into the window Dr. Chilton wants quieted. Lexapro start carries activation risk in the first 2–4 weeks, especially in older adults; Dad's [activation-intolerance pattern across aripiprazole, mirtazapine, brexpiprazole, olanzapine, pramipexole, and lithium](#) makes him a worse-than-average candidate. SSRI anxiolytic effect takes 4–8 weeks — even with the A2 gap, the turbulence window lands before any benefit window does.
- **The switch goes backward in Dad's treatment history.** Escitalopram was the long-running SSRI through ~10/2025 and was replaced by duloxetine because the SSRI wasn't holding the depression. No new evidence that it would work better now than it did 7 months ago. Worth reviewing alongside [handout #09 — Antidepressant three-arm decision aid](#).
- **Removing trazodone and doxepin is the wrong direction.** Both are AGS-Beers-endorsed in older adults at these doses, Chilton-cleared, and Dad-tolerated. See [handout #13](#). Lemborexant alone is reasonable — it's what Julane endorsed and the DORA-class add [handout #19](#) flagged as the next sleep step. Lemborexant as *replacement* for the working stack is a different and worse proposal than lemborexant as an *addition*.
- **Divalproex has no clear stated indication.** Costs in older adults: cognitive effects, sedation, weight gain (in someone already losing weight unintentionally), hepatotoxicity, thrombocytopenia. Dad already filled it once and didn't take it. Lithium was discontinued 4/21 because the mood-stabilizer trial didn't pay off. See [handout #01 — Depakote](#).
- **The post-session coherence is a real ketamine response signal,** not a footnote. "Felt better in the chair" and "had an antidepressant response" are different variables. Ketamine response often shows up over hours to days, not in the chair. Dad's specific IV-responder status (Feb 2026) strengthens the case for getting back to IV at A2 rather than backing off the modality.
- **"Retraumatizing" imports a causal claim about durable psychological harm** that cannot be made in the immediate aftermath of a difficult dissociative experience. Hard ≠ harmful. The post-session coherence is direct evidence against the "actively reinforcing prior trauma" reading.
- **Julane having less voice on meds is structurally backward.** She has continuous longitudinal observation of Dad in functional state, no relationship-management pressure to maintain access, and no prior medication decisions to defend. The proposal pattern recognized in the Seroquel episode (senior provider names a clinical goal → Dr. R assembles an aggressive restructuring in the name of that goal) is exactly why a third perpendicular voice matters.

- **ECT belongs on the table as a contingent backup if a fairly-tested ketamine course doesn't respond**, not as a pivot driven by one rough Day 1 IV session at an unfamiliar setting. See [handout #04 — Ketamine decision aid](#) for prior framing.

RECOMMENDED PATH

1. **Confirm Dr. Chilton's actual scope** before any medication change moves.
2. **Hold the medication stack stable through the A2 gap.** Duloxetine, doxepin, trazodone PRN, melatonin all continue.
3. **Add lemborexant alone if sleep needs further support.** Don't couple to removal of doxepin/trazodone. Julane-endorsed; matches [handout #19](#).
4. **Defer the depakote conversation** until there's an explicit indication.
5. **Use the 2–3 week gap deliberately** for behavioral preparation with Julane and for the NPI top-tier items from [handout #19](#): caffeine cutoff (verify with Dad directly), digital CBT-I start, bright-light parameter check, exercise to 3×/week, alcohol abstinence.
6. **Plan A2 intake with explicit pre-session anxiolysis protocol** — ask the clinic about single-dose lorazepam pre-session, in-room provider presence, set/setting considerations.
7. **Capture all provider communications in writing** going forward — proposed changes, indications, timing.